

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Patient at 32 weeks gestation with medium preeclampsia and macrosomia risk. Elevated systolic BP (142 mmHg) and mild proteinuria (dipstick 1) are noted. Fasting and postprandial glucose levels are also elevated, coupled with a high pre-pregnancy BMI.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Patient ID: TEST_MED_MED, Gestational Age: 32 weeks. Preeclampsia risk tier: Medium (Score: 0.6395). Macrosomia risk tier: Medium (Score: 0.5943). Key Vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. Fasting Glucose 99 mg/dL, Postprandial 1hr Glucose 126 mg/dL. Pre-pregnancy BMI 42.4. No severe headaches, vision changes, upper abdominal pain, or swelling of face/hands reported. The elevated systolic BP (142 mmHg) and presence of proteinuria, in conjunction with the medium preeclampsia risk tier, warrant clinical evaluation. The elevated glucose levels and high pre-pregnancy BMI suggest a need for further assessment and potential management strategies for macrosomia prevention. Recommend prompt review and appropriate management based on clinical judgment.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.